

Ethics and Communication

Answers and explanations

Graham Steell edition · September 2026

32 explanations, matched to the question set by item number.

Each page explains why every wrong option fails.

Answer key

Item 1	B	Item 2	E	Item 3	C	Item 4	A
Item 5	C	Item 6	D	Item 7	D	Item 9	C
Item 11	D	Item 13	A	Item 16	E	Item 17	D
Item 18	C	Item 19	B	Item 20	C	Item 23	F
Item 24	D	Item 25	E	Item 26	D	Item 27	D
Item 28	A	Item 29	E	Item 30	A	Item 31	B
Item 32	D	Item 33	A	Item 34	A	Item 35	D
Item 36	D	Item 38	D	Item 39	B	Item 40	E

Sex is a biologic identity determined by anatomy (eg, external genitalia), chromosomes, and hormones. In contrast, **gender identity** is an internal identity that can be expressed externally (eg, clothing, behavior). Although biologic sex is often used to guide medical diagnosis, particularly for conditions such as abdominal pain, physicians must also recognize gender diversity to provide **culturally competent medical care** to transgender patients.

This begins with accurately identifying the patient's **preferred name and gender identifier**. Physicians should approach questions about gender identity in a **supportive and tactful** manner. Because physical attributes and self-identifiers (eg, clothing) of transgender patients may not be evident or may change over time, a tactful approach uses **open-ended questions** to elicit the patient's preferences. It is helpful to use a routine opening statement (eg, "I like to ask my patients about the name they use and their pronouns") that **normalizes gender diversity**. The patient's preferences should then be **accurately and consistently documented** in the electronic medical record.

(Choices A and C) Although these questions acknowledge gender diversity, they are both closed-ended and too abrupt for an introduction. They also assume a binary male-female paradigm and exclude recognition of a nonbinary gender, in which a person views gender as a spectrum and identifies as neither male nor female.

(Choice D) This approach is open-ended and allows the patient to express the patient's preferred gender identity; however, it is overly frank as an opening question and may alienate the patient. In addition, it hints at discriminatory documentation throughout the medical record, which may erode patient trust and harm rapport.

(Choice E) Knowing the patient's assigned sex at birth is important for accurate diagnosis and treatment; however, this is an uncomfortable initial question and does not clarify the patient's current gender identity or preferred pronouns.

Educational objective:

Gender-diverse patients require culturally competent medical care, which includes a tactful inquiry about preferred identifiers (eg, name, pronouns) followed by accurate and consistent documentation in the electronic medical record.

References

[Sex, gender, and why the differences matter.](#)

[Gender identity and the management of the transgender patient: a guide for non-specialists.](#)

Explanation

Nearly 20% of the United States population speaks languages other than English at home. It is therefore essential that physicians assess language proficiency when delivering care to immigrant populations. This patient's immigrant background and difficulty describing her pain should raise concern about limited English proficiency (LEP) and the need for a qualified medical interpreter. In the United States, patients with LEP experience a variety of health care disparities, including reduced clinical encounter time, substandard medical treatment, and higher rates of adverse events and preventable medical errors. The safety of LEP patients is particularly jeopardized in high-risk clinical situations such as emergency department care, surgical care, hospital discharge, and medical reconciliation.

A useful screening technique to assess language proficiency is to ask the patient, "How well do you speak English: Not at all, not well, well, or very well?" An interpreter is appropriate for patients who cannot respond or respond "not well." Recommended interventions to prevent adverse events include providing a qualified medical interpreter in high-risk clinical situations and providing translated educational materials. Using the "teach back" method (in which patients are asked to "teach" the physician what was just explained to them) can also serve to confirm understanding.

(Choices A, B, and C) Understanding cultural and social factors and sensitivity in obtaining a sexual history are important components of patient-centered care. However, addressing a potential language barrier takes precedence.

(Choice D) Contacting the patient's family is unlikely to be productive and may violate patient confidentiality.

Educational objective:

Physicians must be able to identify patients with limited English proficiency and ensure that professional interpreters are made available in high-risk clinical situations.

References

- [↗ Language proficiency and adverse events in US hospitals: a pilot study.](#)
- [↗ Do professional interpreters improve clinical care for patients with limited English proficiency? A systematic review of the literature.](#)
- [↗ Identification of limited English proficient patients in clinical care.](#)

Explanation

Although most births in the United States occur in a hospital, the number of **planned home births** has steadily increased. Common reasons for choosing home birth include desires to avoid labor interventions (eg, artificial rupture of membranes), have greater personal control in the birth process, and deliver in a familiar home environment.

Notably, most patients who have a planned home birth are **multiparous** and have had a **prior in-hospital delivery**. In some cases, desiring a planned home birth for the current pregnancy may reflect **low satisfaction or negative experience** with the prior in-hospital delivery. Therefore, providers should seek to understand what, if any, aspects of the prior in-hospital birth experience were concerning to the patient. This is also an opportunity to review the risks (eg, increased neonatal mortality, lack of access to emergency medical interventions) and benefits (eg, higher vaginal delivery rate, fewer obstetric interventions) of home birth. Patients determined to proceed with a home birth require counseling on the indications for transfer to a hospital (eg, abnormal fetal heart rate, arrest of labor).

(Choice A) Although home births are associated with increased neonatal mortality, stating that home birth is risky (ie, "I don't think we should consider this option") does not allow the patient to express her concerns and is paternalistic.

(Choice B) Safe home births require more than a good support system (eg, partner, family, doula). Ideally, an experienced home birth provider with appropriate resources (eg, neonatal resuscitation kit) is present, and there is a backup plan for transfer to a hospital if needed. In addition, this statement shifts responsibility for this patient's care to another (nonmedical) provider, which is inappropriate.

(Choice D) This patient's prior vaginal delivery and absence of chronic medical conditions are favorable for home birth. However, the caveat "as long as your pregnancy remains uncomplicated" may set the patient up for disappointment if obstetric complications arise. In addition, it may discourage her from disclosing symptoms (eg, vaginal bleeding, persistent headache) of serious obstetric complications.

(Choice E) Delaying the conversation about home birth dismisses the patient's concerns, which will likely harm the physician-patient relationship. Exploring her reasons for wanting a planned home birth is a more patient-centered approach.

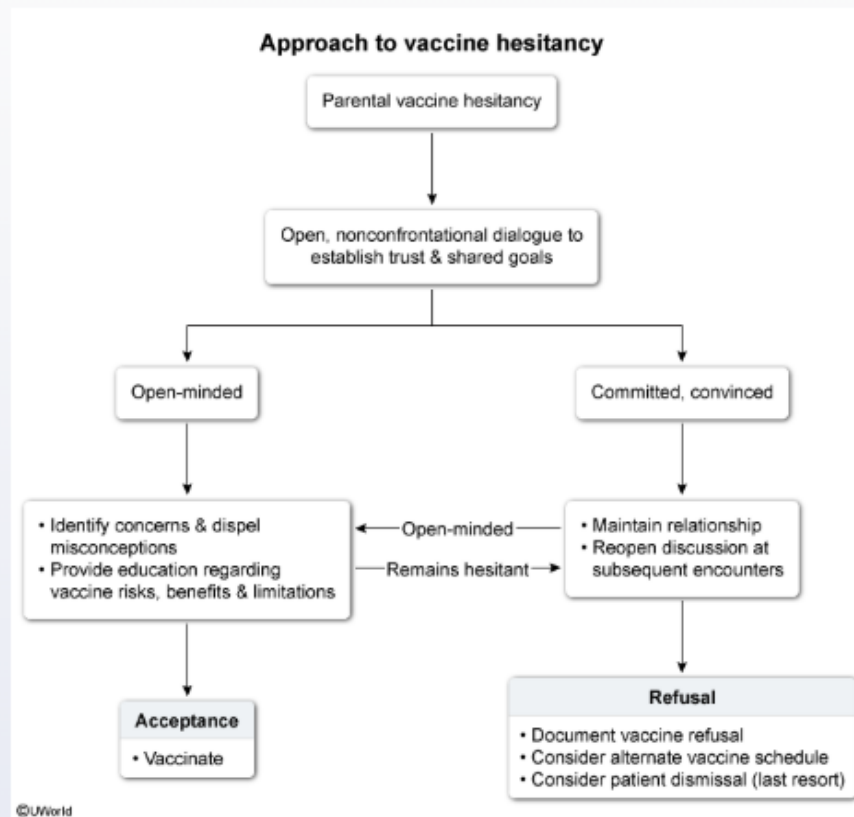
Educational objective:

Desiring a planned home birth may reflect low satisfaction or negative experiences with a prior in-hospital delivery. Therefore, providers should assess what, if any, aspects of the prior in-hospital delivery were concerning to the patient.

References

[Planned out-of-hospital birth and birth outcomes.](#)

Explanation



Vaccine hesitancy is the delay or refusal of immunization. Reasons typically include safety concerns, desire to express autonomy, distrust of medicine, or ethical/religious beliefs. Some parents also believe that natural infection is benign and/or the risk of infection is too low to warrant preventive measures. However, vaccination is critical on an individual and community level to decrease the incidence and severity of vaccine-preventable illness.

The first step in approaching parents who are vaccine-hesitant is to initiate an open and **nonconfrontational dialogue**. The conversation should include:

- relaying the **shared goal** of the parent and provider, which is the **child's health and safety**.
- **acknowledging the parent's concern** as well as the accessibility to vast amounts of vaccine information, both evidence-based and not, that can be overwhelming and confusing.
- **dispelling specific misconceptions**. For example, this patient's mother is concerned that vaccines can be harmful to the immune system, whereas vaccines in fact elicit an immune response to protect from future infection. In addition, some reported symptoms postvaccination (eg, prolonged cough) are often incorrectly attributed to the vaccination but are more likely secondary to a concomitant illness.
- explaining the risks, benefits, and limitations of vaccinations.

In this case, the first step is to establish trust by acknowledging the parent's concern and recognizing a common goal, the well-being of the child.

(Choice B) Although proceeding with vaccination is the objective, this mother has expressed specific concerns that should be addressed first.

(Choice C) Providing a list of evidence-based resources helps educate parents and is recommended when discussing immunizations. However, active dialogue with this parent who is vaccine-hesitant should be pursued first to discuss and correct misinformation.

(Choice D) Reopening the conversation at visits after initial refusal is critical because up to half of vaccine-hesitant parents ultimately choose to vaccinate. However, every opportunity for open dialogue should be taken rather than immediately deferring the conversation to a later time.

(Choice E) Discussing the risks of natural infection is essential in addressing vaccine benefits. However, this mother may perceive her fears about vaccination to be valid if the provider does not dispel her inaccurate beliefs but instead focuses only on the risks of natural infection.

Educational objective:

The first step in approaching parents who are vaccine-hesitant is to initiate an open dialogue that begins with the shared goal of ensuring the child's health and safety. Discussion should also acknowledge parental concerns, correct misinformation, and explain the evidence-based risks and benefits of immunization.

References

- Identifying and addressing vaccine hesitancy.
- Countering vaccine hesitancy.

Explanation

Trust (eg, belief in the physician's skills, recommendations, and intentions) is central to the patient-provider therapeutic alliance and is associated with improved outcomes (eg, treatment adherence, quality, self-reported health). Among **minority patients**, distrust of physicians and health care systems may occur due to the following:

- **Historical legacies** of experimentation (eg, US Public Health Service Syphilis Study at Tuskegee, which failed to obtain proper informed consent from Black men or provide them with curative and readily available treatment)
- Personal experience of **discrimination**, because minority patients are more likely to receive lower-quality health care and to experience suboptimal physician communication
- **Low ethnic diversity** in health care (eg, <5% of physicians are Black; <6% are Hispanic)

Distrust can decrease vaccine use (eg, <40% of Black adults receive the influenza immunization). Given the patient's high risk for influenza complications (eg, history of asthma, diabetes), the physician should explore the reasons for her concerns and engage her in an **open-ended discussion** using **culturally inclusive** health education materials; such materials may feature messages, images, or symbols that carry meaning to a minority community. Discussion should **build a partnership** and establish alignment of patient and physician goals. Once the patient's concerns and priorities are understood, the physician may be able to provide reassurance through patient-centered education.

(Choice A) Although the daughter's encouragement may be helpful, the physician should first attempt to directly engage the patient.

(Choice B) Distrust in the medical system should be respectfully explored rather than viewed as delusional, given its legitimate relation to historical events and personal experience.

(Choice D) Inquiring about the patient's understanding of federally funded health care systems is less focused and relevant compared to direct discussion, exploration of views, and reassurance regarding the influenza vaccination.

(Choice E) Although the patient has the right to ultimately decline vaccination, the physician should first attempt to engage her with open-ended discussion about it.

Educational objective:

Trust is central to the provider-patient therapeutic alliance and is associated with improved outcomes. Distrust by minority patients may result from numerous factors (eg, historical legacy, experience of discrimination, low cultural inclusivity). Providers can build trust through open-ended discussion and focused reassurance using culturally relevant materials.

References

-  Trust in the health care system and the use of preventive health services by older black and white adults.
-  African American adults and seasonal influenza vaccination: Changing our approach can move the needle.

Explanation

Advance care planning (ACP) identifies a patient's personal preferences for health care in case of chronic, serious illness. It is a flexible, patient-centered process (ie, **prioritizes patient values**) that delineates key patient preferences that can then guide medical decision-making. The process may include (but does not necessitate) creation of legal documents, such as appointment of a surrogate decision maker who acts on the patient's behalf if the patient is **incapacitated**. ACP often begins with an informal conversation with the patient (and trusted family members, if appropriate) to explore the patient's beliefs about illness, death, and health care interventions.

The ACP process is meant to promote patient empowerment by ensuring that patients receive care aligned with their expressed values. However, patient participation in ACP remains low (<11% of patients) and occurs even less frequently among patients who belong to ethnic minorities (eg, Hispanic, Black, Asian). Ethnic **disparities in ACP initiation** and completion may result from **distrust** of health care systems, cultural norms, misconceptions, and suboptimal physician communication.

This patient's statement indicates that she has a misconception about ACP—that she must "sign away" her life with legally binding documents. It also indicates that she values ownership over her health care decisions—a value that is in fact supported by ACP. Therefore, the physician should respond by **explaining the ACP shared goal** of patient empowerment (eg, of allowing her preferences to be shared) in simple, nonjudgmental terms. Expressing this patient-centered goal can **increase trust**, acceptability, and likelihood of patient engagement in subsequent discussions.

(Choice A) Given the patient's low probability of 12-month survival, realistic conversations about end-of-life care are important. However, this statement does not clarify the purpose of ACP and focuses on the physician's opinion more than the patient's expressed values.

(Choice B) The patient has not expressed fear, so this statement is less likely to engage her and may appear condescending. Moreover, the statement does not offer clarity on the purpose and benefits of ACP.

(Choice C) This statement alludes to only one potential outcome of ACP: participation of family members in patient care (eg, as surrogate decision makers). The primary purpose of ACP is to identify and honor the patient's wishes; it is premature to assume that the patient values or prefers family involvement.

(Choice E) If the patient continues to decline engagement, her wishes should be respected. However, the physician should first clarify potential misconceptions about ACP (eg, "sign away my life") and attempt to find common ground by highlighting the shared, patient-centered goal of ACP.

Educational objective:

Advance care planning (ACP) is the process of identifying a patient's health care goals in case of serious illness. Patient participation in ACP is low, especially among ethnic minorities due to factors such as mistrust or suboptimal physician communication. Physicians should engage patients in ACP by first explaining its primary purpose of patient empowerment (ie, identifying patient preferences that can subsequently guide medical decision-making).

Medical Library

 [Advance care planning](#)

Explanation

Patient-centered care requires communication skills that engage the patient in discussing his or her perspective of the illness, which helps build rapport and aids in acquiring a more complete history. In this scenario, the wife begins the conversation by expressing her concern about the patient's medication adherence. Although acquiring collateral information can be important, the best approach is to **redirect the conversation back to the patient** to first elicit his perspective and concerns. It is better to explore medication adherence once rapport has already been established.

Medical interviewing is best initiated with an **open-ended question**. This invites the patient to describe their condition in their own words and gives the physician insight into the patient's major concerns, which can be addressed with more specific questions later in the interview. Examples of open-ended questions include: "What brings you in today?" and "What is that like for you?" In contrast, a closed-ended question requires only a yes/no response and does not encourage conversation/exploration.

(Choices A, B, and E) Although the wife has alerted the physician to a potentially important issue (possible medication nonadherence), it would be inappropriate to begin a medical interview with questions that assume (without asking) that the patient is, in fact, not taking his medication and focus on the wife's concerns rather than the patient's. This may put the patient on the defensive, leading to a deterioration of the physician-patient relationship. In addition, asking the patient if he is having trouble with remembering his medications is an example of a yes/no, closed-ended question.

(Choice C) Although patients should be given the opportunity to speak to the physician alone, this question suggests the wife should leave. It is preferable to ask the patient if he would like her to stay or leave.

Educational objective:

Patient-centered medical interviewing should focus on the patient's perspective of the illness. Interviews are best initiated with an open-ended question that elicits the patient's major concerns in the patient's own words.

References

[🔗 Patient-centered communication: basic skills.](#)

Explanation

Patient-centered care involves being responsive to the patient's expressed concerns and providing necessary information to assist the patient in making an informed health care decision. The risks and benefits of hormone therapy for menopausal symptoms is a complex topic that requires an individualized approach, taking into account the patient's age, severity of symptoms, and calculated risks for breast cancer and cardiovascular disease. Strategies for effective communication include **acknowledging** the **patient's discomfort** and **educating** the patient about the **benefits and risks** and alternate strategies. The emphasis should be on **sharing decision-making**, respecting patient preferences, and avoiding premature judgments.

In this case, the patient has expressed significant distress and has specifically asked if hormone therapy can alleviate her discomfort. The most appropriate response is to acknowledge the impact of the patient's vasomotor symptoms and begin a discussion of the risks and benefits of hormonal treatment in language that the patient can understand.

(Choice A) This response fails to promote shared decision-making by dismissing the patient's inquiry about hormone therapy. The patient should be educated about the benefits and risks of hormone therapy so that she can make an informed medical decision.

(Choice B) This patient has come to the physician with the expectation that her discomfort can be alleviated. This response both minimizes the patient's discomfort and offers no relief. The patient will likely feel that her concerns have not been addressed, negatively impacting the patient-physician relationship.

(Choice D) This response makes a judgment about the patient's level of discomfort and inappropriately makes the decision for her without conducting an informed consent discussion about the risks and benefits.

(Choice E) Although discussing coping strategies can be helpful, this response redirects the conversation and ignores the patient's question regarding hormone therapy.

Educational objective:

Patient-centered care involves being responsive to the patient's expressed concerns and providing necessary information to assist the patient in making an informed health care decision. The emphasis should be on shared decision-making, taking into account the patient's preferences and avoiding premature judgments.

References

[Cardiovascular risk/benefit profile of MHT.](#)

Explanation

This physician is confronted by a patient who has not experienced benefit from previous treatment and **feels hopeless** that his condition will improve. Such challenging patients can often cause the physician to feel defensive, manipulated, or hopeless as well, leading to premature referral to a specialist.

Although strategies to improve treatment adherence will be important for improving outcomes, the best initial approach is to **build the physician-patient relationship** by **empathizing** with the **patient's frustration** and **disappointment** with past treatments. In this way, the patient will feel understood rather than blamed for past treatment failures, and he will be more willing to work with the physician to develop a collaborative plan focused on realistic goals.

(Choice A) This patient requires a thorough history and physical examination to assess his condition. Referral to a pain specialist is premature.

(Choice B) This statement is likely to be experienced as unsupportive because it challenges the patient's proposal regarding disability and does not offer any suggestions in its place.

(Choice C) This statement attempts to point out the patient's help-rejecting pattern of behavior, but it would likely be perceived as judgmental, especially at this early stage of the physician-patient relationship.

(Choice E) Although at some point the physician should address the importance of adherence to treatment in achieving a better outcome, this statement is critical and blames the patient for past treatment failures. Empathizing with the patient's frustration at this early stage would be more effective in developing a treatment alliance.

Educational objective:

Physicians may become frustrated with a patient who is hopeless about treatment, often compelling the physician to refer the patient to a specialist prematurely. Empathizing with the patient's frustration over past treatment failures can help build the physician-patient relationship.

References

[Managing difficult encounters: understanding physician, patient, and situational factors.](#)

Explanation

Approach to treatment refusal	
Determine legal decision-maker	• Self: legal/competent adult or emancipated minor • Surrogate decision-maker: parent/guardian, specified proxy, default surrogate (eg, spouse, adult child)
Counsel decision-maker	• Assess level of understanding, health literacy • Explain natural history of disease, pros & cons of treatment options
Continued refusal	• For life-sustaining treatment: ◦ Engage care team, social services ◦ Consult facility ethics committee ◦ Initiate legal action for court order • If immediate care is needed (eg, life- or limb-threatening condition): ◦ Provider should administer care over surrogate refusal

This child with a history of recurrent otitis media now has acute mastoiditis, a purulent infection of the mastoid air cells. **Untreated mastoiditis** can lead to serious complications, including **brain abscess and death**, and requires aggressive management. Initial treatment includes parenteral antibiotics, and patients often require surgical intervention.

Patients and surrogate decision-makers (eg, parents, guardians) often have limited understanding of serious but uncommon conditions, such as mastoiditis. They also may have misconceptions regarding the potential risks and benefits of medical treatment. Decision-making can be further complicated by the urgency to make life-or-death decisions quickly in unfamiliar environments (eg, emergency department).

The initial step in counseling patients and surrogate decision-makers regarding **refusal of treatment** is to calmly probe their **understanding of the disease process**. This can be followed by **counseling and education** tailored to the decision-maker's **level of understanding and health literacy** and an objective assessment of treatment options. Although a court order may be necessary to compel the parents to allow life-saving treatment, legal action can be distressing to all involved. Often, once the disease process and treatment options are better understood, decision-makers become less resistant to treatment.

(Choice B) For non-life-threatening conditions for which standard therapies are undesirable or ineffective, clinicians can lend their expertise to patients regarding nonstandard therapies, even if the clinician does not fully endorse such therapies. However, there are no nonpharmacologic therapies that are adequate for treating mastoiditis.

(Choice C) This statement begins with an appropriate gesture of concern. However, it is inadequate because it simply reiterates treatment recommendations and does not attempt to provide the information needed for the patient's parents to make an informed decision.

(Choice D) Although discussion with this child's family should include the risks of untreated mastoiditis, presenting scare tactics and worst-case scenarios is likely only to further distress the family and inhibit rational decision-making.

(Choice E) Delayed treatment of mastoiditis is not recommended. Wasting time with ineffective treatments will only delay having a more appropriate conversation.

Educational objective:

Patients and surrogate decision-makers often have misconceptions regarding medical conditions and the potential risks and benefits of treatment. The initial step in counseling patients and surrogate decision-makers regarding treatment refusal is to calmly probe their understanding of the disease process. This can be followed by counseling and education tailored to the decision-maker's level of understanding and health literacy.

Medical Library

 [Informed consent in medicine](#)

Explanation

Motivational interviewing	
Indications	• Substance use disorders • Other behaviors in patients who are not ready to change
Principles	• Acknowledge resistance to change • Address discrepancies between behavior & long-term goals • Enhance motivation to change (support self-efficacy) • Remain nonjudgmental
Technique (OARS)	• Ask Open-ended questions (encourage further discussion) • Give Affirmations • Reflect & Summarize main points

This patient says he feels "like a failure" because he used cocaine again following a 3-month period of abstinence. Focusing on the negative consequences of cocaine use (which the patient is already aware of) may be perceived as shaming and judgmental. The best approach is to help the patient feel understood by acknowledging his feelings of disappointment but **reframing** his relapse as a **temporary setback** he can surmount.

Principles of **motivational interviewing** can be used to help this patient get back on track and strengthen his belief that he can keep making progress. Motivational interviewing is a **nonjudgmental, collaborative**, patient-centered approach that **enhances** the patient's sense of self-confidence and **self-efficacy**. By emphasizing the patient's past success in maintaining abstinence for a period of time, the physician supports the patient's belief in his ability to learn from this setback and make changes. Open-ended discussion of what contributed to using cocaine again from the patient's perspective can then be used to guide the patient to make a specific plan to achieve and implement change.

(Choice A) This question has a judgmental tone and fails to acknowledge how the patient feels. It also assumes the patient will keep using and prematurely focuses on possible negative consequences of continued use.

(Choice B) This approach also focuses on reiterating the negative health effects of cocaine rather than addressing the patient's loss of confidence in his ability to make changes. Focusing on his strengths rather than on negative consequences is more likely to build internal motivation.

(Choice C) Although this statement seems supportive, it discourages self-reflection and does not help build the patient's sense of self-efficacy.

(Choice D) Rather than first exploring the patient's own thoughts about his relapse, this approach is assumptive and reflects a desire to control the agenda while rushing to a premature solution. Such a statement misses the opportunity to elicit the patient's perspective and help build his confidence in his own ability to make changes.

Educational objective:

Principles of motivational interviewing can be used in the treatment of substance use disorders to guide responses to patients who relapse. This technique helps build patients' sense of self-efficacy by emphasizing their past successes in maintaining abstinence and by supporting patients' belief in their ability to make changes.

Behavioral science
Subject

Social Sciences (Ethics/Legal/Professional)
System

Motivational interviewing
Topic

Explanation

Trust (ie, belief in the physician's recommendations and intentions) is central to the physician-patient **therapeutic alliance** and necessary for optimal outcomes. Distrust may arise from prior negative health care experiences, such as adverse events (eg, diagnostic error, treatment failure) or overt or perceived physician **bias** (eg, against **uninsured** patients).

This patient with signs of posttraumatic peritonitis (eg, rebound tenderness, guarding) expresses reluctance to undergo surgery in a way that signals distrust (ie, concern that she is being treated as a guinea pig). To ensure that the patient receives appropriate treatment and an optimal outcome, the physician should apply the following principles to build trust and promote treatment acceptability:

- Acknowledge trust as a central therapeutic goal.
- **Find common ground** (eg, identify a shared goal of feeling better).
- Seek **patient perspectives** and prioritize patient-centered care (ie, treatment aligned with patient values).

(Choice A) This statement unnecessarily focuses the discussion on the physician's perspective (eg, compensation) without addressing the patient's concern about being a guinea pig or exploring her underlying reasons for refusing surgery.

(Choice B) This statement reflects empathy for the patient's suffering but is fear based (ie, focuses on negative consequences). It is unlikely to promote acceptance in a patient whose reason for refusing surgery is low trust; in fact, the patient could perceive this statement as coercive or threatening.

(Choice C) This question assumes that the patient's refusal is based on financial concerns; her statement is more indicative of distrust.

(Choice E) Attributing this patient's response to pain ignores her central concern of trust and may be viewed as condescending.

Educational objective:

Trust is central to the physician-patient relationship, and distrust may arise from prior negative experiences (eg, adverse events, bias). Physicians can build trust by identifying shared goals, seeking patient perspectives, and prioritizing patient-centered care.

Behavioral science
Subject

Social Sciences (Ethics/Legal/Professional)
System

Physician patient communication
Topic

Explanation

Stages of change model		
Stage		Motivational interviewing
Precontemplation	Not ready to change: patient does not acknowledge negative consequences	• Encourage patient to evaluate consequences of current behavior • Explain & personalize the risk • Recommending action is premature
Contemplation	Thinking of changing: patient acknowledges consequences but is ambivalent	• Encourage evaluation of pros & cons of behavior change • Promote new, positive behaviors
Preparation	Ready to change: patient decides to change	• Encourage small initial steps • Reinforce positive outcome expectations
Action	Making change: patient makes specific, overt changes	• Help identify appropriate change strategies & enlist social support • Promote self-efficacy for dealing with obstacles
Maintenance	Changes integrated into patient's life; focus on relapse prevention	• Follow-up support; reinforce intrinsic rewards • Develop relapse prevention strategies
Identification	Behavior is automatic: changes incorporated into sense of self	• Praise changes

This patient has obesity with weight-related complications including hypertension and (likely) type 2 diabetes mellitus. However, she does not acknowledge the adverse effects of obesity and voices no readiness to institute behavioral changes (eg, diet, exercise). Counseling of patients with obesity and other lifestyle-related disorders (eg, smoking, alcohol abuse) can often be tailored to the individual patient by using a stages of change model; this patient is considered to be in the **precontemplation stage**.

Patients in the precontemplation stage are not ready to explore lifestyle change options or implement specific behavioral interventions. However, it is often beneficial to explore the patient's **current understanding** of their condition and how it can affect their **future health**. Patients with obesity frequently understand much about the potential health consequences; engaging in a nonjudgmental discussion of how this patient's weight affects her blood pressure and glucose may facilitate more productive conversations in future visits.

(Choice A) Although bariatric surgery is indicated based on this patient's BMI, noninvasive treatments (ie, diet, weight loss medication) are usually attempted first. Also, specific interventions are generally not discussed until the patient is ready to consider options for change.

(Choice B) Once a patient voices willingness to consider lifestyle change (ie, contemplation stage), the clinician should start introducing potential interventions in general terms, discussing the various pros and cons. However, in this case, the clinician should first encourage the patient to consider the need for lifestyle change before discussing possible interventions.

(Choice D) Many diets (eg, low-fat, low-carbohydrate, reduced-calorie blended) have been shown to produce weight loss, but most require changes in food choices, not just food quantity. Details of implementation generally would not be discussed until the patient voices some level of readiness to change.

(Choice E) Initiating a weight loss program with small, incremental goals is frequently helpful in the implementation of lifestyle change. However, setting goals is more appropriate at the time the patient is preparing to institute behavior change (ie, preparation stage).

Educational objective:

Patients in a precontemplation stage of change do not acknowledge the negative consequences of current behaviors and are not ready to implement specific lifestyle changes. Providers working with patients in the precontemplation stage should avoid recommending specific actions but instead discuss patients' understanding of their condition and its effect on their future health. A nonjudgmental discussion of lifestyle can often facilitate more productive conversations in the future.

References

Explanation

The sudden, **unexpected loss** of a loved one produces intense, acute grief. When the cause of death is suicide, the emotional pain may be further complicated by a range of difficult emotions including guilt and anger as well as difficulty comprehending what happened. The need to make sense of the suicide may be especially intense when the family member perceives that the patient was doing better beforehand, as in this case.

After a patient's death, **disclosure of pertinent medical history** (including psychiatric history) is permitted to a personal representative (eg, spouse, parent) under the Health Insurance Portability and Accountability Act (HIPAA). The immediate **postsuicide response** should **acknowledge** the bereaved individual's severe **emotional distress** and offer to **answer questions** related to clinical course and treatment that may help to make sense of what happened. This discussion should be conducted in an open-ended fashion, without making any assumptions about how the bereaved individual is feeling or what they want to know. It is particularly important to help alleviate any sense of inappropriate guilt that the bereaved individuals may be experiencing and make them aware of additional resources to assist with suicide bereavement if needed (eg, grief counseling, suicide bereavement support groups).

(Choice A) Inquiring if the patient was taking his medication as prescribed inappropriately focuses the conversation on trying to find out specific reasons for the suicide rather than responding to the wife's acute, emotional distress in a compassionate manner that allows for open discussion.

(Choice C) This response attributes the suicide to medication changes when it is unclear to what extent, if any, this was a contributing factor.

(Choice D) This statement empathizes with the wife's shock but asks her to share details of her husband's death, which she may not feel comfortable doing. It is preferable to allow the wife to express her grief and ask any questions she may have about her husband's illness and treatment.

(Choice E) Asking the wife if she noticed anything unusual in the week leading up to the suicide may have the unintended consequence of making her feel that she missed signs and is in some way to blame.

Educational objective:

Responding to bereaved individuals following a suicide requires a compassionate and open-ended approach. Clinicians should acknowledge the bereaved individual's emotional distress and offer to answer any questions pertaining to clinical course and treatment that may help to make sense of what happened.

Behavioral science
Subject

Social Sciences (Ethics/Legal/Professional)
System

Suicide
Topic

Explanation

Motivational interviewing	
Indications	• Substance use disorders • Other behaviors in patients who are not ready to change
Principles	• Acknowledge resistance to change • Address discrepancies between behavior & long-term goals • Enhance motivation to change • Nonjudgmental
Technique (OARS)	• Ask Open-ended questions (encourage further discussion) • Give Affirmations • Reflect & Summarize main points

This patient has a family history of lung cancer and multiple risk factors for cardiovascular illness, including smoking, hypertension, hyperlipidemia, and family history of stroke. Although he states that he is **not ready to quit smoking**, he is ambivalent and aware that he probably should. An effective approach to patients who are not ready to quit is to **explore their ambivalence** (eg, worries about quitting vs not quitting), with the goal of **enhancing** the patient's **internal motivation** to make a change.

Based on principles of motivational interviewing, the physician should acknowledge resistance to change and help the patient identify discrepancies between habits and long-term goals. Issues relevant to the patient's situation (eg, concern about his ability to manage work stress without smoking) and other possible barriers (eg, fear of nicotine withdrawal, failure, or gaining weight; enjoyment of tobacco) should be explored in a nonjudgmental way. Concerns about not quitting, such as fear of long-term health risks, can then be elicited to highlight ambivalence. Once the patient has identified the potential benefits and barriers to quitting, the physician can target these specific issues (eg, education, counseling, medication).

(Choice A) This response redirects the conversation to the benefits of quitting and fails to acknowledge or explore the patient's resistance to change and his personal barriers to quitting.

(Choice B) Although this statement acknowledges the patient's stress and conveys optimism about his ability to quit, it fails to address his ambivalent feelings, which may cause the patient to feel misunderstood and judged for his reluctance to quit.

(Choice D) This response explores the patient's understanding of specific health risks of smoking but misses the opportunity to develop the patient's internal motivation to quit.

(Choice E) The physician should first explore the patient's personal concerns and barriers to quitting before discussing smoking cessation treatment options.

Educational objective:

For patients who are not ready to quit smoking, it is helpful to explore their perspective about the pros and cons of continuing to smoke. Acknowledging resistance in a nonjudgmental manner while exploring ambivalence can help develop patients' internal motivation to change.

Medical Library

 [Smoking cessation](#)

 [Stages of behavioral change](#)

Explanation

Overutilization	
Definition	Services (tests or treatments) that: • Are not reasonably expected to benefit the patient • Are not necessary for clinical decision-making
Strategies to reduce overutilization	• Determine how a test will affect management before ordering • Use evidence-based practice guidelines that emphasize services with proven benefit • Avoid redundant tests & treatment

This patient has acute, uncomplicated low back pain (LBP) with no "red-flag" features (eg, fever, neurologic deficits) to suggest a serious cause. Uncomplicated LBP usually resolves spontaneously, and management is directed toward providing temporary symptomatic relief. Imaging (eg, x-ray, MRI scan) is generally not recommended because it does not improve outcomes or provide necessary information for optimal management.

Overutilization refers to provision of **unnecessary services** that are not expected to improve **patient outcomes** or meaningfully change **management**. Patient-centered health care requires care to be delivered with respect to the patients' concerns, preferences, and priorities, but does not require unnecessary testing or treatment if patient input is otherwise solicited and respected when appropriate. MRI in uncomplicated LBP increases overutilization and is therefore not recommended for this patient.

(Choice A) Spinal MRI often identifies nonspecific abnormalities (eg, bulging intervertebral discs) that are not relevant to management but can lead to further unnecessary and potentially harmful diagnostic studies and interventions. Because of the frequency of such abnormalities, even in asymptomatic individuals, it is often not clear whether they are the actual cause of pain or merely an incidental finding. Deferring imaging therefore decreases, rather than increases, diagnostic uncertainty.

(Choice B) Fragmentation of health care refers to breaking up care across multiple providers and facilities without adequate cross communication. Declining to order unnecessary tests does not lead to fragmentation.

(Choice C) Health care disparity refers to differences in access to care or outcomes due to socioeconomic or demographic factors. Careful and appropriate use of diagnostic studies does not increase disparity.

(Choice D) Malpractice liability risk correlates most closely with provider-patient communication and perceived level of concern. Appropriate use of diagnostic studies, especially as it conforms to evidence-based guidelines, does not increase malpractice risk.

(Choice E) Clinical standardization refers to consistency of management decisions within and across provider groups. Care that conforms to established guidelines increases, not decreases, standardization.

Educational objective:

Overutilization refers to provision of services that are not expected to improve patient outcomes or meaningfully change management. Patient-centered health care requires care to be delivered with respect to patients' concerns, preferences, and priorities, but does not require unnecessary testing or treatment if patient input is otherwise solicited and respected when appropriate.

References

- [Managing inappropriate utilization of laboratory resources.](#)
- [Mechanical low back pain.](#)

Explanation

Low literacy	
Risk factors	• Low level of completed education • History of incarceration • Low socioeconomic status • Language difference between patient & provider
Clinical clues	• Has multiple visits for the same condition • Asks family or friends to read medical literature • Refuses to fill out paperwork
Adverse health outcomes	• Treatment nonadherence • ↑ Hospitalizations & emergency care use

This patient's cholesteatoma requires surgical intervention; lack of definitive treatment has led to multiple emergency department visits for the same condition. During today's office visit, although the patient seeks medical care, he **does not** appear to **engage with the physician** (eg, nods briefly but asks no questions) and does not read the written health information sheets but instead hands them to his companion. This presentation raises **suspicion for low literacy**, a diminished ability to read and write.

Low literacy has been shown to correlate with many **adverse health outcomes**, including increased hospitalizations and emergency care use, decreased adult vaccinations (eg, influenza), poorer self-reported health status, and increased mortality. Low literacy may be difficult to detect because known risk factors (eg, low level of completed education, history of incarceration) may not be present. In addition, adults are often ashamed and may try to **conceal** low literacy, sometimes with apparent noncompliance or refusal to cooperate.

Screening reading tests (eg, asking the patient to read the information sheet aloud) are typically discouraged because they may induce anxiety and/or shame (**Choice C**). However, questions such as "How often do you have other people help you interpret medical documents?" or "How confident are you in filling out medical forms?" may reveal low literacy in a supportive fashion. If a physician suspects that low literacy is contributing to poor patient outcomes, alternative forms of communication such as **visual resources** (eg, illustrations, videos) should be used.

(Choice A) Although adults with low literacy may require additional educational strategies (eg, visual aids) to gain appropriate understanding before making health care decisions, it should not be assumed that they lack decision-making capacity. This patient is likely a capable adult; therefore, further communication should be directed toward him rather than a family member.

(Choices B and E) Explaining the condition again or verbally reviewing instructions for scheduling the procedure is unlikely to improve the outcome because many individuals with low literacy experience challenges in both written and spoken communication. Using an alternative mode of education that includes visual learning is more likely to improve comprehension.

Educational objective:

Low literacy can be a significant barrier to appropriate treatment and may be difficult to identify. For patients with suspected low literacy, alternative methods of communication (eg, visual resources) should be used to improve understanding.

Behavioral science
Subject

Social Sciences (Ethics/Legal/Professional)
System

Physician patient communication
Topic

Explanation

In this scenario, the physician has an **initial encounter** with an **angry patient** who is upset about an aspect of her care that is not under the physician's direct control. Nevertheless, it is the physician's responsibility to be **responsive** to the **patient's immediate concerns**, attempt to defuse her anger, and build the physician-patient relationship. The physician should acknowledge the cause of the patient's distress (ie, hunger), remain nondefensive, and offer to obtain further information that could **help resolve the issue** (eg, confirming correct diet was ordered; checking with the nursing staff).

The physician should maintain a professional demeanor and avoid responding to the patient's anger with comments that may be interpreted as dismissive, defensive, or sarcastic. Being responsive to the patient's immediate concerns is more likely to build rapport, help the patient feel understood and lay a positive foundation for the physician patient relationship.

(Choice A) This statement acknowledges the patient's distress but makes no attempt to respond to her concern.

(Choice B) This statement offers an apology but inappropriately reassures the patient that the problem has been resolved when this is unknown.

(Choice C) This statement may be interpreted as defensive because it focuses on explaining that the problem is not the physician's responsibility. Quickly addressing the patient's needs is more likely to defuse the situation and build the physician-patient relationship.

(Choice D) Deferring resolution of the issue until after the examination is unlikely to defuse the patient's immediate anger and could, in turn, negatively affect her cooperation with the examination.

Educational objective:

Initial encounters with angry patients are best managed by acknowledging their distress and responding to their immediate concerns.

References

[Defusing the angry patient: 25 tips.](#)

[Managing difficult encounters: understanding physician, patient, and situational factors.](#)

Behavioral science
Subject

Social Sciences (Ethics/Legal/Professional)
System

Physician patient communication
Topic

Explanation

This patient has biopsy-confirmed **non-small cell lung cancer** (NSCLC). However, the prognosis (ie, likely course or outcome of disease) cannot be determined because **prognosis** in NSCLC is heavily **dependent on disease stage**, and the necessary staging studies (eg, CT scan of the abdomen) have not yet been completed.

At this time, the physician should **discuss all available information** with the patient. Even though prognostic information is not yet available, the physician should not withhold diagnostic information, especially when the patient makes a **direct inquiry**. Because prognosis is uncertain and possibly unfavorable, the physician should take a sensitive, patient-centered approach to avoid conveying either a sense of false hope (**Choice C**) or needless hopelessness. The physician should also discuss the plan for completing the staging evaluation and the expected timing for informing the patient of results.

(Choice A) Although deferring discussion of prognosis is appropriate because staging studies have not been completed, the physician should inform the patient of his biopsy results because that information is available and the patient desires to know.

(Choice B) Deferring certain details of the discussion to the oncologist (eg, specific treatment regimen) is appropriate; however, the physician in charge of the patient's care should answer the patient's question and provide all the information that is currently available.

Educational objective:

The physician should inform the patient about all relevant information, particularly when specifically asked by the patient. If only partial information is available, the physician should provide the information available at the time, along with a discussion of when the remaining information will be available.

Behavioral science

Subject

Social Sciences (Ethics/Legal/Professional)

System

Physician patient communication

Topic

Explanation

Pain is subjective, and physicians must use clinical judgment to **balance** effective **pain management** with **prevention of drug overuse** and misuse. Prescription opioid misuse has become an increasing occurrence in recent years. Signs of misuse include not taking medication as prescribed, requesting early refills, accessing drugs from multiple physicians or illicit sources, requesting higher doses, and refusing alternate pain-management strategies.

This patient is showing signs suggestive of opioid use disorder: running out of medication early; obtaining prescriptions in emergency departments rather than from the primary prescriber; tolerance (ie, requiring higher doses to attain the same effect); and dependence. However, confronting the patient about misuse at this point is unlikely to be helpful. The most appropriate initial action is a patient-centered approach: **validating the patient's concern** about pain control; engaging the patient in a nonjudgmental, **collaborative discussion** of how he is using the medication; and **exploring reasons** for the **escalating use**.

(Choice A) This statement may be interpreted as confrontational and put the patient on the defensive. It fails to validate the patient's subjective experience of pain and instead focuses on the physician's concerns about dependence.

(Choices B and E) These statements reject the patient's request for more medication without discussion and prematurely shut down the conversation. This approach is likely to frustrate the patient, making it less likely that he will be receptive to alternate approaches to pain management.

(Choice C) This response prematurely switches the focus of the conversation to mood, ignoring the patient's concerns. Raising the possibility of self-medication of mood symptoms would be more appropriate once the patient has already engaged in a collaborative discussion of how he is using the medication.

Educational objective:

When misuse of prescription opioids is suspected, physicians should first engage patients in a nonjudgmental, collaborative discussion to understand the reasons for the misuse.

References

- [Prescription opioid misuse, abuse, and treatment in the United States: an update.](#)
- [Prescription opioid abuse in chronic pain: an updated review of opioid abuse predictors and strategies to curb opioid abuse: part 1.](#)

Explanation

Communicating with patients who have limited English proficiency	
Initial consent	- Determine patient's primary/preferred language - Obtain consent for interpreter services
Preferred mode of communication	Interpreter (or provider) fluent in both medical communication & patient's preferred language
Alternate modes*	- Telephonic or remote interpreter service - Nonprovider staff (eg, medical assistant, receptionist) - Family/friends - Written communication
*Not recommended if medical interpreter can be obtained & patient consents.	

This patient requires surgical intervention and must give appropriate consent before proceeding. However, he has **limited English proficiency** (LEP) and will require language assistance to make informed decisions about his health care. Best practices for medical communication with a patient who has LEP is to use an interpreter (or provider) trained in medical translation in the patient's primary language. Using a **trained medical interpreter** can help avoid potentially dangerous miscommunication, and leads to fewer clinical errors, higher patient satisfaction, and better clinical outcomes. However, using an interpreter or other personnel to provide care is subject to **consent by the patient**.

If a face-to-face interpreter is unavailable, video or telephonic remote translation services should be offered. If the situation is urgent and there is no time to wait for interpretation services, communication should be facilitated by any tools available (eg, friends/family, writing/drawing instruments, bilingual hospital staff); however, these options should not be considered standard procedure in nonemergency situations.

(Choice B) Printing out a Vietnamese language informed consent form would be helpful. However, it does not replace the need for a medical interpreter who can provide details of the procedure and guarantee that the patient's questions are answered thoroughly.

(Choices C and D) Friends and family members are not ideal for language interpretation due to lack of impartiality and possible barriers to open communication regarding sensitive topics (eg, domestic abuse). They may also be unfamiliar with medical terminology and may struggle to accurately convey complex medical information (although this patient's son has medical training). This patient should be offered the services of a trained medical translator; family would be used only if the patient declines an outside translator.

(Choice E) Although this statement reflects a potential reason to avoid using family members as interpreters, there is nothing in the family interaction to suggest a lack of objectivity, and such a direct response may be taken as demeaning or insensitive.

Educational objective:

Physicians must ensure the appropriate use of medical interpreters to promote adequate patient understanding and participation in the decision-making process. This is particularly important when obtaining informed consent for treatment.

References

[Interpreters in health care: a concise review for clinicians.](#)

Explanation

This patient's difficulty following written directions and reluctance to fill out paperwork are clues that suggest **low health literacy**, a common and underrecognized barrier to health care. Low health literacy (ie, difficulty with accessing, understanding, or using medical information to make decisions) contributes to poor health care access and outcomes, including medication errors, missed appointments, and failure to follow up with tests and referrals.

Although formal literacy tests have been used in research, they are not recommended in clinical settings because they can be stigmatizing. Effective **communication** requires a patient-centered approach that **avoids causing shame** and embarrassment. Using an open-ended question that **encourages discussion of any concerns** about making the appointment is the best way to address the issue without shaming the patient. If this patient expressed difficulty, he could be assisted in scheduling the appointment while in the office.

(Choice A) How far someone went in school does not necessarily indicate literacy level. In addition, this may be perceived as judgmental and condescending.

(Choice B) Many patients with low literacy levels are embarrassed and attempt to conceal their difficulty. This confrontational approach assumes the patient has difficulty reading and writing, which may make him increasingly anxious and uncomfortable.

(Choice C) This response inappropriately puts the patient on the spot to assess his literacy level.

(Choice D) This response ignores the clues that suggest possible low health literacy. Failing to address this increases the risk that the patient will not schedule the cystoscopy and be lost to follow-up.

Educational objective:

Low health literacy is a common and underrecognized barrier to health care. It is important to assess patients' understanding of provided information without shaming or causing embarrassment.

References

[Health literacy in primary care practice.](#)

Explanation

Sexual history is a key part of comprehensive patient care and should be obtained at all initial visits, routine preventive screenings, and visits when sexually transmitted infections are suspected. Physicians should attempt to put patients of all sexual orientations and gender identities at ease when reviewing **sexual history** because, for some, the discussion may feel uncomfortable and intrusive. Normalizing the discussion as a routine component of the medical examination, ensuring confidentiality, remaining nonjudgmental, and expressing a willingness to address all sexual health concerns can facilitate the discussion.

When asking about sexual partners, the physician should **avoid** using labels or making **assumptions** about patients or their partners' sexual orientation or gender identity. Asking an **open-ended** question about **all sexual partners** allows patients to describe their sexual partners and behavior in terms that they are familiar with and that reflect their gender identity. After inquiring about the gender of sexual partners (including trans, nonbinary), the physician can ask about pertinent types of sex (eg, vaginal, oral, anal) with each partner in order to make testing recommendations for sexually transmitted infections (eg, HIV) and discuss risk-reduction strategies. Another possible formulation is, "What is/are the sex and gender of your sexual partner(s)?"

(Choice B) This question is nonspecific and does not directly address sexual behavior. An individual may be sexually active outside of a romantic relationship or may not view an interaction as romantic. Although this question may yield some information regarding the patient's relationship history, it may exclude important information about some partners and sexual behaviors.

(Choice C) Questioning the patient about other sexual partners while he was married may make him feel judged and less willing to disclose his sexual history. This inquiry also fails to focus on all sexual partners.

(Choice D) This question forces the patient to choose a sexual orientation label. Many individuals may identify as heterosexual but have sexual contact with individuals of the same gender. Others may not relate to any of these labels to describe their sexuality.

(Choice E) Asking the patient whether he has ever had intercourse with a man is a narrow, close-ended question that does not address the possible range of his sexual behaviors or partners.

Educational objective:

Taking a sexual history is a key part of comprehensive care and requires an inclusive and nonjudgmental approach. Physicians should avoid making assumptions about a patient's sexual history and use an open-ended inquiry about all sexual partners.

References

- [Sexual health history: techniques and tips.](#)
- [Sexual orientation and patient-provider communication about sexual problems or concerns among US adults.](#)
- [Best practices in North American pre-clinical medical education in sexual history taking: consensus from the Summits in Medical Education in Sexual Health.](#)

Explanation

Sexual history is an essential part of a patient's medical history but is a topic that may be uncomfortable for both patients and physicians. The best approach is to try to put the patient at ease by **normalizing the sexual history** as a **routine component** of the medical history. The physician's demeanor should convey that the sexual history is routinely obtained and necessary to provide good health care, similar to other key parts of the physician-patient interaction (eg, obtaining substance use history, physical examination) that might be experienced as intrusive in other contexts. By inquiring about sexual history in the same clinical and empathetic manner used for other aspects of a history and physical, physicians can place a patient at ease and create a trusting space in which a patient can talk about potentially uncomfortable and intimate topics.

(Choice A) It is inappropriate to defer this critical aspect of the history to a later date based on an unverified assumption of patient discomfort. The patient's responses may indicate the need for additional STD testing, which should not be deferred.

(Choice C) This statement has a judgmental tone and implies that the patient's lack of openness might negatively impact her treatment. It is unlikely to facilitate open communication.

(Choice D) Although it may be appropriate to explore a patient's discomfort with a particular topic, this question is premature because it relies on the unverified assumption that the patient is uncomfortable with the topic, which may make her defensive. Normalizing sexual history and explaining why it is important is more likely to be productive.

(Choice E) It would be inappropriate to offer a diagnosis before completing a full evaluation and workup.

Educational objective:

Sexual history is an essential part of gathering a complete medical history. This topic may be uncomfortable for both patient and physician, so it is important to normalize it as a routine part of a medical evaluation.

Behavioral science

Subject

Social Sciences (Ethics/Legal/Professional)

System

Physician patient communication

Topic

Explanation

This patient has experienced **sexual assault**. Many patients who have been sexually assaulted do not seek medical attention. This hesitancy is typically due to fear of judgment with **disclosure**. Because physicians are often the first line of support for survivors of sexual assault, their response to the disclosure has a significant impact on patient comfort during the conversation and examination.

The initial statement made after a patient discloses sexual assault should be **supportive**, nonjudgmental, and patient centered, with emphasis on **promoting safety and trust** (eg, "it's important that you feel safe here"). Because sexual assault can make the survivor feel powerless (loss of control), the opening statement should also **empower patients** by allowing them to guide the evaluation and **set the pacing** (eg, "please let me know if you need to stop or take a break at any point."). Direct questioning about the assault and its effects, which can be perceived as threatening or result in flashbacks and retraumatization, should be avoided initially (**Choice C**).

(**Choice A**) Although a pelvic examination may be indicated (eg, forensic evidence collection, sexually transmitted disease testing), it should not be the focus of the initial statement after patients make a vulnerable disclosure of sexual assault. Adequate time should be spent reassuring safety and allowing patients to direct the pacing of the evaluation. In contrast, recommending that patients undergo a pelvic examination without first setting them at ease and discussing the indications and benefits may lead to retraumatization.

(**Choice B**) Emergency contraception is important to address following sexual assault; however, it should be discussed with the patient after establishing rapport, performing the examination, and obtaining appropriate testing (eg, urine pregnancy test).

(**Choice E**) Although trauma can affect memory, the most likely cause of this patient's hazy memory is the assailant's use of a mind-altering drug. In addition, the statement is dismissive of the patient's reason to not file a police report, and it may make her feel pressured to involve the police or feel guilty if she chooses not to.

Educational objective:

Physicians are often the first line of support for survivors of sexual assault. Disclosure of sexual assault should be met with supportive, nonjudgmental, and patient-centered statements. Patients are encouraged to set the pacing of the evaluation.

References

- [Responding to intimate partner violence and sexual violence against women: WHO clinical and policy guidelines.](#)
- [Communication tips for caring for survivors of sexual assault.](#)

Behavioral science
Subject

Social Sciences (Ethics/Legal/Professional)
System

Sexual assault
Topic

Explanation

When patients & families request that "everything" be done	
Understand what "everything" means	• "Every treatment with even the slightest possibility of benefit" (rare) • "Everything that you, as my physician, feel is worthwhile" • "Everything to relieve symptoms, even though it may shorten life"
Communicate prognosis	• Use clear & direct language ("Your mother is dying") • Maintain consistent messaging • Avoid discussing minutiae of management
Propose a philosophy of treatment	• Offer medical recommendations with appropriate limits to therapy • Discuss what <i>will</i> be done before covering what will <i>not</i> be done • Illustrate how the recommendations support the patient's values
Support emotional response	• Validate emotions & invite conversation ("What is the toughest part of this for you?") • Reaffirm commitment to caring for the patient no matter what happens
Negotiate disagreements	• Review understanding & seek common ground • Seek external input for persistent, unresolved conflict (eg, from ethics committee)

This gravely ill patient has acute respiratory failure due to refractory metastatic lung cancer. Because the patient is rapidly deteriorating, discussions regarding **goals of care** are time-sensitive, as she may soon be incapacitated (eg, urgently intubated) and unable to participate in her own decision-making (**Choice E**).

The patient and her family request that **"everything"** be done, a frequently voiced sentiment during care discussions. In the outpatient oncology setting, "everything" meant exhausting multiple chemotherapy regimens to achieve a disease response. Unfortunately, the patient has become unstable and is very likely to die during her acute hospitalization. Predefined preferences should therefore be **revisited regularly**, including whenever there is a **major change** in clinical status. The physician cannot presume that a patient's past preferences can be directly extrapolated to a **new situation**.

Patients can be unsure of how to express **evolving preferences** without an open invitation to a discussion. Therefore, clarifying the patient's current concept of "everything" is the first step toward developing a shared plan of care. "Everything" has fluid meanings, shifting across both a patient's life span and different clinical situations. Due to this wide variability, it is critical to guide and support patients as they weigh the acceptability of potentially harmful interventions with a low likelihood of benefit.

(Choice B) The patient and her oncologist may have previously discussed long-term goals of care. However, her current preferences need to be revisited in light of her acute deterioration, with specific clarification about the meaning of "everything."

(Choices C and D) Some patients request "everything to keep me alive at all costs." Radiation and airway stenting can temporarily improve pulmonary function and extend this patient's life by days to weeks. However, her personal beliefs, values, and concept of "everything" should be openly discussed and reevaluated prior to offering treatments that may ultimately prolong the inevitable dying process with no meaningful benefit.

Educational objective:

When approaching goals of care, the specific meaning of a patient or family's request that "everything be done" should be explored and revisited regularly, including whenever there is a major change in clinical status.

Behavioral science

Subject

Social Sciences (Ethics/Legal/Professional)

System

Medical futility

Topic

Explanation

Hearing loss is common, impacting almost 20% of US adults; the prevalence increases with age. Hearing loss is associated with increased rates of misdiagnosis, longer lengths of hospitalizations, and increased readmission rates. As such, physicians should employ strategies to improve communication, which include:

- **Decreasing background noise**
- **Facing the patient directly** when speaking and refraining from touching the face or mouth
- **Speaking clearly and loudly** (ie, natural amplification) without shouting or shrieking, which distorts the sound of speech
- **Speaking at a normal pace** because speaking too slowly can decrease contextual clues and make it hard for patients to understand
- Using short and simple sentences and addressing one topic at a time
- Using facial expressions, gestures, and visual aids when appropriate
- Asking the patient to repeat back specific instructions to ensure understanding

Hearing loss patients are **heterogeneous** with varying needs. Physicians should ask open-ended questions such as, "What is the best way for me to communicate with you?" and frequently ask for feedback to ensure that communication is effective. In addition, physicians should **document successful strategies** to improve future encounters.

(Choice B) Although this patient has hearing loss, he is able to verbally communicate effectively with the physician and does not need to write. The physician (rather than the patient) may use written communication if this is in line with the patient's preferences. In general, writing is not preferred because nuances can be lost, intentions misinterpreted, or information left out for the sake of brevity.

(Choice C) Obtaining history from the wife or next of kin is appropriate if the patient is incapacitated (eg, comatose state). Attempts to communicate directly with the patient are always preferred.

(Choice D) Speaking loudly into one ear may be helpful for some patients with unilateral hearing loss. However, this should only be done after confirming this aligns with the patient's preferences and asking which ear is best for hearing (ie, the better-hearing ear).

(Choice E) Optimal care of patients with hearing loss who use sign language requires trained interpreters. However, many patients with hearing loss do not use sign language; this assumption can lead to delays in care and embarrassment for the patient.

Educational objective:

Hearing loss can impair communication, resulting in adverse health outcomes. Strategies to improve communication include minimizing background noise, facing the patient directly, and speaking clearly at a normal pace.

References

[Improving communication with patients with a hearing impairment.](#)

Behavioral science
Subject

Social Sciences (Ethics/Legal/Professional)
System

Physician patient communication
Topic

Explanation

Outpatient office schedules are typically arranged in **regular time intervals** (eg, 15-20 min) corresponding to the time needed for an average patient visit. Typically, the time interval allows the physician to evaluate **one primary objective**, either evaluating a new problem or following up on well-known chronic conditions.

Sometimes, patients raise **unexpected concerns** that require additional time beyond what is allotted for their appointments; evaluation of this patient's urinary symptoms would extend his appointment, which has already been spent addressing his chronic conditions. Extending the appointment without having adequate time would make **subsequent patient visits late** and can lead to **rushed and incomplete assessments** (for this and/or subsequent patients), potentially resulting in misdiagnosis, mismanagement, or poor patient-physician communication (**Choice A**).

Therefore, physicians should generally direct the patient with an unexpected concern to **schedule another appointment** to address it; doing so allows the physician to fully address the concern without causing delays in subsequent appointments. However, when the unexpected concern is urgent or likely **serious** (eg, shortness of breath), it should be addressed **immediately**, and subsequent patients should be informed of the delay.

(Choice B) This patient has a health maintenance examination (ie, routine physical) in a month. Evaluating new concerns in the time allotted for preventive care is usually not recommended because it may impact the quality of care for preventive services. Instead, a dedicated appointment should be scheduled.

(Choice C) Querying the patient at the beginning of the visit regarding any unexpected concerns can help the physician prioritize the discussion. However, the patient should not be dismissed at the end of the appointment without making arrangements to address his concerns properly.

Educational objective:

When a patient raises an unexpected concern during an office appointment and adequate time has not been allotted to evaluate it, the physician should generally ask the patient to schedule an appointment at a later date to address that concern.

Behavioral science
Subject

Social Sciences (Ethics/Legal/Professional)
System

Professional conduct
Topic

Explanation

Building a therapeutic physician-patient relationship requires finding an optimal balance between friendliness and professionalism. First impressions are important, and **initial encounters** with new patients often set the tone for the ensuing relationship. Using the patient's **preferred form of address** can help build rapport. Clarifying any uncertainties about pronunciation, titles, or pronouns is important as well. Although younger adults often prefer being called by first name, older patients may take offense at what they consider undue familiarity. When in doubt, the physician should ask patients their preferred form of address.

As familiarity develops between the physician and patient, many patients will prefer to be addressed by first name. However, it should be the patient, not the physician, who takes the lead in lowering the level of formality. It is better to ask the patient's preference in an open-ended manner rather than make assumptions.

(Choices A and B) Although the patient has addressed the intern by his first name, it is still inappropriate to assume that she wants to be addressed by her first name. The patient's preferred form of address should be clarified first.

(Choice C) This patient has expressed positive feelings toward the intern because he reminds her of her son. Using her surname without clarifying if that is her preference shifts the tone of the interaction and may be perceived as overly formal and distant. Asking her preferred form of address would be best.

(Choice E) Initiating an interview by instructing or correcting the patient in front of the medical team, no matter how gently or politely, may embarrass the patient and result in deterioration of rapport.

Educational objective:

Initial encounters with new patients often set the tone for the physician-patient relationship. When in doubt about how to address patients, the physician should ask them their preferred form of address.

References

- [Building trust and rapport early in the new doctor-patient relationship: a longitudinal qualitative study.](#)
- [Medical communication: the views of simulated patients.](#)

Explanation

Patients commonly use **herbal** preparations and **dietary supplements**. Although the US Food and Drug Administration (FDA) regulates dietary supplements, the regulations are less strict than those for prescription and over-the-counter medications. Some herbal products have established safety and/or efficacy, but there are **few quality assurances** regarding the content and purity of many herbal supplements, and unknown toxic ingredients may cause **adverse effects** and dangerous **drug interactions**. For example, the herbal supplement ephedra, marketed as a diet pill and energy booster, was banned by the FDA in 2004 due to the risk of cardiovascular complications (eg, increased blood pressure, myocardial infarction, stroke). However, ephedra-containing products are still readily available through the Internet.

It is important for physicians to **routinely inquire** about the use of these products, both to understand why patients are using them and to evaluate the risks and benefits of continued use. This patient should be supported in her weight-loss efforts and her use of supplements should not be immediately dismissed. The physician should instead **assess the ingredients** for contraindications or drug interactions and then provide **counsel regarding the potential risks**, especially given her medical history.

(Choice A) Although the physician should work with the patient to develop a comprehensive weight-loss plan, including behavioral approaches to weight loss, this response is dismissive and does not take into account the use of supplements, which this patient is likely to continue taking.

(Choices B and E) Supporting the patient's continued use of the supplement or merely cautioning her to avoid additional caffeine, without assessing the supplement for dangerous ingredients such as ephedra, does not adequately address safety concerns.

(Choice C) The patient may interpret this response as discouraging her efforts to lose weight, negatively impacting rapport with the physician. In addition, it does not address the risks or benefits of the supplement itself.

Educational objective:

Physicians should routinely ask their patients about the use of herbal preparations and nutritional supplements and advise them on the quality, safety, and efficacy of these products.

References

- [Reduction in ephedra poisonings after FDA ban.](#)
- [Overview of regulation of dietary supplements in the USA and issues of adulteration with phenethylamines \(PEAs\).](#)
- [An overview of herb and dietary supplement efficacy, safety and government regulations in the United States with suggested improvements. Part 1 of 5 series.](#)

Explanation

Communicating the news of a suspected diagnosis of **Down syndrome** (DS) at the time of delivery requires an **empathic approach** that is responsive to the parent's emotional needs and **avoids negative bias** about the diagnosis. Parents should be informed of the diagnosis as soon as possible in a private setting that allows them time to process the news, express their emotions, and ask any questions they may have. Initiating the discussion with statements that frame the diagnosis negatively (eg, "I'm sorry," "I have bad news") should be avoided because they convey implicit bias that the intellectual disability associated with DS is devastating, which may not reflect the parents' values or views about their child. A shift from "breaking bad news" to "sharing unexpected news" can help prepare the parents to receive the information without imposing negative value judgments.

Detailed information about the diagnosis should be tailored to the parents' knowledge base and how much they want to know at this point in time. The physician should provide information about local resources and support groups and make a plan for follow-up discussion.

(Choice A) This statement uses medical jargon to deliver life-changing news without warning. Parents should be prepared that unexpected news is coming, and the physician should use language that is easily understood.

(Choices C and D) These statements are not neutral and frame the diagnosis negatively. Initiating the discussion with "unexpected" rather than "bad" news is preferable.

(Choice E) Sharing alarming news that the baby's examination is abnormal and inquiring if prenatal testing was done is insensitive and may be perceived as judgmental. When characteristic dysmorphic features of DS are present, it is better to share the findings, explain the diagnosis, and answer any questions the parents may have.

Educational objective:

Communicating the news of a suspected diagnosis of Down syndrome at the time of delivery requires an empathic approach. A shift from "breaking bad news" to "sharing unexpected news" can help prepare the parents to receive the information without imposing negative value judgments.

References

-  [Practice guidelines for communicating a prenatal or postnatal diagnosis of Down syndrome: recommendations of the National Society of Genetic Counselors.](#)
-  [When bad news isn't necessarily bad: recognizing provider bias when sharing unexpected news.](#)

Behavioral science

Subject

Social Sciences (Ethics/Legal/Professional)

System

Physician patient communication

Topic

Explanation

Motivational interviewing	
Indications	• Substance use disorders • Other behaviors in patients who are not ready to change
Principles	• Acknowledge resistance to change • Address discrepancies between behavior & long-term goals • Enhance motivation to change (support self-efficacy) • Remain nonjudgmental
Technique (OARS)	• Ask Open-ended questions (encourage further discussion) • Give Affirmations • Reflect & Summarize main points

This patient is struggling to quit smoking and is discouraged by repeated failures. Rather than repeat the health risks of smoking or prematurely focus on treatment, the physician can be more productive by using motivational interviewing to build and support the patient's motivation to change. **Motivational interviewing** is a patient-centered approach that invites collaboration, helps **develop internal motivation** to change, and enhances the patient's sense of optimism, self-confidence, and self-efficacy in reaching goals.

The physician's most appropriate response is to support the patient's sense of **self-efficacy** by helping her reflect on her **past success** and apply it to her current difficulties. By reviewing successful methods that enabled the patient to stop using cocaine, the physician can build the patient's internal motivation and belief that she can change her smoking habits as well.

(Choices A and D) This patient has been counseled about smoking and has tried to quit several times. She is likely aware that stopping smoking will improve her breathing and overall health. Rather than express concern and repeat the risks of smoking, the physician would be more effective by eliciting the patient's perspective and exploring her concerns about the effects of smoking on her breathing.

(Choice B) Although this statement is empathic, it prematurely focuses on treatment without attempting to elicit the patient's strengths and build her intrinsic motivation.

(Choice C) This statement is supportive and optimistic but reflects the physician's confidence, not the patient's. It does not help develop the patient's sense of self-efficacy.

Educational objective:

Motivational interviewing supports and develops patients' sense of self-efficacy in making behavioral changes. Eliciting patients' strengths and focusing on past successes can help build patients' internal motivation and confidence in their ability to make changes.

Behavioral science
Subject

Social Sciences (Ethics/Legal/Professional)
System

Motivational interviewing
Topic